SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Nivolumab, pemetrexed and cisplatin before surgery

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Neoadjuvant NSCLC · selected pemetrexed / cisplatin backbone · NCCP 00849 · version 2 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment is given before planned surgery for resectable non-small-cell lung cancer at high risk of returning. It aims to treat the cancer before the operation and reduce the chance of recurrence. Surgery and benefit are not guaranteed.

Schedule
Up to three 21-day cycles. Day 1: nivolumab, pemetrexed and cisplatin. Then surgical MDT reassessment. The five listed backbones are alternatives; this guide covers only your selected combination.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Urgent symptoms and local contact details confirmed
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Support medicines and phase changes explained
- ☐ Common, serious and lasting drug-specific risks discussed below

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Nivolumab
IMMUNOTHERAPY

- ☐ Immune inflammation in multiple organs
- ☐ Infusion reactions
- ☐ Delayed effects after treatment stops

Pemetrexed
ANTIFOLATE CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Mouth soreness and diarrhoea
- ☐ Kidney problems
- ☐ Allergic or infusion reactions
- ☐ Skin rash; rarely serious skin reactions
- ☐ Rare serious lung inflammation

Cisplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Mouth soreness and diarrhoea
- ☐ Kidney problems
- ☐ Allergic or infusion reactions
- ☐ Hearing changes or tinnitus, which can persist
- ☐ Numbness or tingling, which can persist
- ☐ Electrolyte changes and need for hydration

Take the prescribed folic acid, vitamin B12 and rash-prevention steroid plan; continue folic acid for 21 days after the final pemetrexed dose. Cisplatin needs hydration, electrolyte checks and monitoring of kidneys, hearing and nerves. Nivolumab can inflame healthy organs during treatment or after it stops. Report new symptoms promptly and carry your immune alert card for at least five months after the last dose. Discuss severe or fatal immune inflammation and chemotherapy effects, including lasting hormone replacement, organ injury and effects on surgery timing.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00849 v2.

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